

HOW TO OPEN AN ASSISTED LIVING HOME FOR ADULTS WITH BRAIN INJURY IN TEXAS

A Texas HHSC-licensed assisted living facility (Type A or Type B) serving adults with brain injury — with the optional Comprehensive Rehabilitation Services (CRS) post-acute rehabilitation path.

A step-by-step startup and operations guide

Licensing (TULIP) · Manager & staff training · Funding · Optional CRS contracting · Billing

Regulatory basis: 26 TAC Chapter 553 (Licensing Standards for Assisted Living Facilities); Texas Health & Safety Code Chapter 247; optional CRS pathway: CRS Standards for Providers and 1 TAC §355.9040.

How to Use This Guide

This guide walks you through opening and operating a home in Texas that provides residential care to adults living with brain injury. That home is licensed by the Texas Health and Human Services Commission (HHSC) as an assisted living facility (ALF) under 26 TAC Chapter 553 and Texas Health & Safety Code Chapter 247. There is one licensing path — you get the home licensed as a Type A or Type B ALF and operate it — and one honest funding reality: Texas has no standalone long-term brain-injury Medicaid waiver, so the home runs on private pay and each resident's own benefits, with an optional add-on — contracting with HHSC's Comprehensive Rehabilitation Services (CRS) program for time-limited post-acute rehabilitation. This guide treats the ALF as the core and CRS as the optional revenue path.

Important — verify before you act

This guide is an educational resource, not legal, financial, tax, or regulatory advice. Requirements, forms, fees, and rates change. Confirm every step with HHSC (Long-Term Care Regulation / Licensing and Credentialing) and, for the optional path, the HHSC CRS program, before you act. Texas Medicaid does not pay for ALF room and board; the dedicated brain-injury funding (CRS) is time-limited post-acute rehab, not indefinite long-term residential — see Parts 7 and 9.

Part 1 — What This Business Is

A Texas brain-injury residential home is an assisted living facility licensed by HHSC under 26 TAC Chapter 553. An ALF furnishes food and shelter to four or more people unrelated to the proprietor and provides personal care services and/or medication assistance or administration. Texas licenses by resident evacuation capability, not by size:

- **Type A** — residents who can evacuate without physical staff assistance, do not require routine attendance during nighttime sleeping hours, and can follow directions under emergency conditions.
- **Type B** — residents who may require staff assistance to evacuate and may require attendance during nighttime sleeping hours.

(Type C, an older four-bed adult-foster-care license, is no longer issued.) Choose the type that matches the residents you intend to serve; brain-injury residents whose cognitive or physical status affects evacuation typically point you toward a Type B license.

Cognitive-care note. Because brain injury commonly affects cognition and behavior, Texas requires all staff in a non-Alzheimer's-certified ALF that serves residents with Alzheimer's or a related disorder to complete competency-based cognitive-care training (26 TAC §553.254). Plan for it.

Part 2 — Who Regulates You

- **HHSC** — Texas Health and Human Services Commission, Long-Term Care Regulation (Licensing and Credentialing). Licenses your ALF, sets the Chapter 553 standards, and conducts the Life Safety Code and health surveys. The HHSC Architectural Unit reviews construction and plans.
- **DFPS** — Texas Department of Family and Protective Services. Receives and investigates reports of abuse, neglect, and exploitation (with HHSC under a memorandum of understanding).
- **Optional — HHSC CRS program and CRS counselors.** If you add the post-acute rehab path, you contract with CRS and take referrals authorized by CRS counselors (Part 9).

Part 3 — Business and Insurance Setup

- Form your business entity with the Texas Secretary of State and be in good standing; be in good standing with the Texas Comptroller of Public Accounts — HHSC verifies both.
- Secure the home (lease or purchase) and confirm local zoning permits a residential care use at the address.

- Obtain liability and other insurance appropriate to an assisted living operation.
- Identify all owners and controlling persons — you disclose them on the application, and staff undergo background and registry checks.
- Obtain a National Provider Identifier (NPI) if you will bill any payer.

Part 4 — The Home: Physical Plant and Fire Safety

Your building must meet the Chapter 553, Subchapter D physical-plant and Life Safety Code (NFPA 101) standards for your license type. Before you request your Life Safety Code (LSC) survey, confirm:

- ☐ The building meets Subchapter D construction and NFPA 101 requirements for your license type and size.
- ☐ Resident sleeping rooms, bathrooms, and common dining/activity areas meet the standards; a safe, accessible layout for residents using walkers or wheelchairs.
- ☐ Fire protection — smoke detection and alarms, and sprinklers where required for the type and size — tested and documented.
- ☐ A written emergency preparedness and response plan, with documented drills; registration with the 2-1-1 Texas disaster system.
- ☐ Your residents' evacuation capability matches your license type (Type A vs. Type B).
- ☐ Safe hot-water temperature; chemicals and hazardous items stored to prevent resident access.
- ☐ A fire marshal inspection report and a letter to the local health authority (both required for the application).

Part 5 — Step 1: Your Manager and Required Training

Texas does not issue a separate assisted-living manager license, but the manager (who may be you) must meet Chapter 553's requirements:

- Be on duty at least 40 hours per week and manage only this facility (a small Type A manager may cover up to 16 residents in up to four facilities), per §553.253(a).
- Complete at least one 24-hour management course, including eight hours on the assisted living standards within the first three months (§553.253).
- Complete the required pre-survey, computer-based training (CBT) and keep the certificate — HHSC requires it for the ALF application.
- Designate, in writing, a competent employee to act in the manager's absence.

Tip: knock out the pre-survey CBT and the manager's course early — HHSC expects the CBT certificate with your application.

Part 6 — Step 2: Get Your HHSC Assisted Living License

Applications are submitted online through **TULIP** (the Texas Unified Licensure Information Portal). **Form 3720** is the Application for State License to Operate a Long-Term Care Facility (Form 3720-G for a Type A/B in good standing).

1. Register in TULIP and start the ALF application (Form 3720).
2. Upload the required documents: tax ID, property documents, letter to the local health authority, fire marshal inspection report, NPI (if applicable), management company agreement (if applicable), your Life Safety Code inspection request, the pre-licensure CBT certificate, and your health readiness letter — plus your Chapter 553-compliant policies and procedures.
3. Pay the fee: initial Type A/B is **\$300 plus \$15 per bed**, with a **\$2,250 maximum for a three-year license** (verify current).
4. Provide written notice the building complies with Subchapter D and is ready for the Life Safety Code (LSC) survey; pass it.

5. Admit at least one but no more than three residents, then notify HHSC you are ready for the health survey; pass it.
6. On a complete application and passing LSC and health surveys, HHSC issues the license within 45 days.

Where your manual and forms fit. The application requires complete Chapter 553 policies and procedures. Your Policy & Procedure Manual and companion Forms Packet are built to that requirement — have them ready before you apply.

Part 7 — Step 3: How You Get Paid — Funding the Home

This is where Texas differs from states with a single brain-injury waiver. Texas has no standalone long-term brain-injury Medicaid waiver that pays for ALF residential care, so plan your core funding around:

- **Private pay** — the resident or family pays for room, board, and care under your residency agreement.
- **The resident's own benefits** — Supplemental Security Income (SSI), long-term-care insurance, VA benefits, and, for some residents, assisted living services delivered through a Medicaid managed-care program such as STAR+PLUS (the plan and service coordinator authorize any covered services; this pays for services, not your room and board).

Note: Texas Medicaid does not pay for ALF room and board. Confirm each resident's coverage — and what it will and will not pay — before admission. The optional path that funds dedicated brain-injury rehabilitation is the CRS add-on in Part 9.

If a resident is on STAR+PLUS: the one code-based avenue

If a resident is enrolled in STAR+PLUS (Texas Medicaid managed care) and receives assisted living as a covered HCBS service, that service is billed to the member's managed care organization (MCO) — not paid privately. The service code is **HPCPS T2031** ("Assisted living; waiver, per diem"), billed per day with modifiers under the STAR+PLUS LTSS billing matrix (Appendix XVI of the STAR+PLUS Handbook).

- The MCO pays for the assisted-living service only — room and board remains the member's responsibility, set at the SSI federal benefit rate minus an \$85 personal needs allowance.
- As of September 1, 2025, HHSC moved this service from the RUG-III model to the Patient-Driven Payment Model (PDPM), rolling out as each member's service plan renews. Confirm the current code, modifiers, and units in Appendix XVI before billing — claims filed with the old codes are denied.
- The rate is set through your MCO contract under the PDPM methodology, not a single public per-diem.

Part 8 — Staffing, Caregivers, and Training

- Manager (Part 5) on duty 40 hours/week; a designated, authorized employee covers the manager's absence.
- Attendants are at least 18 or high-school graduates (§553.253(b)).
- Night-shift coverage by license type: small Type A — staff immediately available; large Type A — immediately available and awake; Type B — immediately available and awake regardless of bed count (§553.253(a)(4)). Texas sets no fixed staff-to-resident ratio; staff to your residents' acuity.
- Screening: criminal history, Nurse Aide Registry, and Employee Misconduct Registry checks before resident contact; suspend an employee reported to the EMR during appeals (§553.257).
- Training: the required rights, abuse/neglect, confidentiality, emergency-response, and cognitive-care topics (§553.253, §553.254), plus brain-injury competencies — cueing and compensatory strategies, wander and fall prevention, and responding to escalated behavior.

Part 9 — The Optional CRS Add-On: Post-Acute Rehabilitation Residential Services

Texas's dedicated brain-injury funding is the Comprehensive Rehabilitation Services (CRS) program, run by HHSC for people with a traumatic brain injury (or TBI with traumatic spinal cord injury). You can add it as a revenue path on top of your ALF.

How it works

- **Licensure prerequisite.** To provide CRS post-acute rehabilitation in a residential setting you must be licensed as an assisted living facility (CRS Standards for Providers, §13.6.1). A new facility provider has up to two years from its CRS contract to obtain the required accreditation.
- **Time-limited and rehab-focused.** Residential CRS services are limited to 180 days from the first day of service and aim at independence — post-acute rehab, not indefinite long-term residence.
- **Team-based.** An interdisciplinary team assesses the participant (MPAI-4 or FIM) and builds an Individualized Program Plan with measurable goals; core therapies include physical, occupational, speech, and cognitive therapy and behavior management.
- **Payer of last resort.** Participants must first use available insurance, Medicare, or Medicaid; CRS pays the copay or coinsurance as provided by rule.

How residential CRS is billed — the tier structure

Residential CRS is billed on a preauthorized tiered structure. Each tier is a set number of core-therapy hours the interdisciplinary team may deliver per week (Sunday through Saturday):

Tier	What it authorizes (preauthorized core-therapy hours per week)
Tier Base	Base residential services only; no core therapy that day (rarely approved).
Tier Base Plus	Limited core therapy, about 0.25–0.75 hour per day (rarely approved).
Tiers 1–6	Ascending preauthorized bands of weekly core-therapy hours. Example: a participant approved at Tier 4 may receive up to 28 hours of core therapy in a week.

- Counting rule: one hour of individual therapy, or two hours of group therapy, counts as one hour toward the tier; group therapy is capped at ten participants.
- Record the tier and services provided each day; submit billing within 30 days of the last date of service and upload service records to the CRS Data Reporting System by the tenth working day of the following month.
- HHSC negotiates provider contracts and reimburses in accordance with 1 TAC §355.9040.

The dollar rate — an honest number, not a made-up one

Unlike states with a single published per-diem, Texas does not publish one CRS residential rate to quote here. The dollar amount for each tier is set by HHSC Provider Finance and confirmed in your CRS provider contract. To give you an accurate figure rather than an invented one: confirm your tier rates in your executed CRS contract and against the current HHSC Provider Finance CRS rate schedule (set under 1 TAC §355.9040 and updated periodically). Because residential CRS is capped at 180 days, plan the rehabilitation arc and discharge within that window.

Part 10 — Timeline, Costs, and Getting Your First Residents

Rough sequence

Form your entity and secure/prepare the home → complete the pre-survey CBT and your manager's course → complete the TULIP application (Form 3720) with policies, documents, and fee → pass the Life Safety Code survey → admit one to three residents and pass the health survey → receive your license (within 45 days of a complete

application and passing surveys) → line up funding (private pay/benefits) and, optionally, contract with CRS. Verify current timeframes with HHSC.

Typical startup costs to budget

- The home (lease or purchase) and any modifications, furnishings, and life-safety systems.
- The manager's 24-hour course and staff training; background checks; CPR and first aid.
- Insurance and business setup; the HHSC license fee (\$300 + \$15/bed, \$2,250 maximum for a three-year license — verify current).
- Your Policy & Procedure Manual and Forms Packet.

Getting your first residents

- **Houston:** build referral relationships with the Texas Medical Center hospitals and post-acute rehabilitation programs (such as TIRR Memorial Hermann), hospital discharge planners, and brain-injury support networks; if you add CRS, connect with the regional CRS counselors who authorize post-acute referrals.
- **Dallas-Fort Worth:** work with the Metroplex's rehabilitation hospitals and trauma centers, discharge planners, and county aging-and-disability resources; the CRS referral channel is the same statewide.

In both markets, private-pay families seeking specialized brain-injury residential care and post-acute rehab discharge planners are your primary referral sources.

Your Compliance Foundation

Opening the home is the first milestone; staying compliant with Chapter 553 is the ongoing work. Two companion resources are built for that: a Policy & Procedure Manual covering the Chapter 553 standards (and the optional CRS section) and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone an HHSC survey expects.

Before you rely on any step in this guide

Verify current requirements, forms, fees, and rates with HHSC (Long-Term Care Regulation / Licensing and Credentialing) and, for the optional path, the HHSC CRS program. Texas Medicaid does not pay ALF room and board, and CRS residential services are limited to 180 days — confirm each resident's funding before admission.